

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the administration of Nitrofurantoin or Trimethoprim for the treatment of UTI in women aged 16–64 years

Summary of NICE / NICE CKS Guidelines for UTI in Non-Pregnant Women (Aged 16–64)

Overview

- UTIs are **common** in women and usually caused by **Escherichia coli**.
- Most are **lower UTIs (cystitis)**, presenting with **dysuria, frequency, urgency, and suprapubic discomfort**.
- Can often be managed **empirically in primary care**.

Diagnosis

Treat empirically if 2 or more of the following symptoms are present:

- Dysuria
- New nocturia
- Urinary frequency

Consider urine dipstick or culture if:

- Only 1 symptom
- Atypical presentation
- Recurrent UTI
- Antibiotic failure
- Suspected pyelonephritis or upper UTI (e.g. fever, loin pain, nausea)

Do not routinely send urine for culture in otherwise healthy women with typical symptoms and no red flags.

Empirical Antibiotic Treatment (First-Line Options)

Antibiotic	Dose	Duration
Nitrofurantoin (if eGFR ≥ 45 mL/min)	100 mg MR BD	3 days
Trimethoprim (if low resistance risk)	200 mg BD	3 days
Pivmecillinam	400 mg initial, then 200 mg TDS	3 days
Fosfomycin	3 g single dose sachet	One dose

Choose based on **local resistance**, **allergies**, and **renal function**.

Avoid trimethoprim in areas with **>20% resistance**, unless culture confirms sensitivity.

Non-Antibiotic Advice

- Encourage fluid intake
- Analgesia (e.g. paracetamol or ibuprofen) for pain
- Symptoms often improve within **48 hours** even without antibiotics
- Seek medical advice if symptoms worsen or persist

When to Send Urine for Culture

- Recurrent UTIs
- Atypical symptoms
- No response to empirical antibiotics
- Known or suspected resistance
- Haematuria or suspected pyelonephritis

When to Refer or Escalate

- **Signs of upper UTI** (fever, flank pain, vomiting)
- **Sepsis features**
- **Persistent symptoms** despite treatment
- **Recurrent UTI** (≥ 2 in 6 months or ≥ 3 in 12 months)
- Anatomical abnormalities or suspected underlying cause

Prevention of Recurrent UTI

- Offer self-care advice (hydration, voiding after sex, avoiding irritants)
- Consider:

- **Antibiotic prophylaxis** (e.g. nitrofurantoin 50–100 mg or trimethoprim 100 mg at night)
 - **Non-antibiotic measures** (e.g. cranberry products, D-mannose) – limited evidence
 - Consider **referral** if recurrent or complicated UTI
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Patient Group Direction

for the supply/administration of

Nitrofurantoin (Nitrofurantoin 100 mg modified-release capsules)

for the treatment of

Urinary Tract Infection (UTI) in women aged 16–64 years

Healthcare professionals covered and training

Patient Group Direction for the administration of Nitrofurantoin for the treatment of UTI in women aged 16- 64	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Nitrofurantoin is indicated for the treatment of uncomplicated lower urinary tract infection (UTI) in non-pregnant women aged 16 to 64 years, confirmed by symptoms such as dysuria, frequency, urgency, and suprapubic discomfort.
Inclusion criteria	- Women aged between 16 and 64 years. - Presenting with typical symptoms of uncomplicated lower UTI (e.g., dysuria, urgency, frequency). - Able to give informed consent. - No signs of systemic infection (e.g., fever, flank pain). - Not currently pregnant or breastfeeding.
Exclusion criteria	- Known hypersensitivity to nitrofurantoin or any excipients in the formulation. - Creatinine clearance <45 mL/min. - History of recurrent UTIs or symptoms lasting more than 7 days. - Presence of fever, flank pain or systemic symptoms suggestive of upper UTI or pyelonephritis. - Pregnant or breastfeeding. - Known hepatic dysfunction or pulmonary disease related to nitrofurantoin.
Cautions	- Consider possibility of sexually transmitted infections if symptoms are atypical or there is vaginal discharge. - Monitor for gastrointestinal intolerance or allergic reaction. - Counsel patients to take the medicine with food and complete the full course.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Nitrofurantoin 100 mg modified-release capsules
Legal category	POM
Storage	Store in original packaging at room temperature below 25°C
Route/method of administration	Oral, to be taken with food
Dose and frequency	100 mg twice daily for 3 days
Quantity to be administered and/or supplied	6 capsules (3-day course)
Maximum or minimum treatment period	3 days; reassess if no improvement or if symptoms recur
Adverse effects	Nausea, headache, flatulence, diarrhoea, and urine discolouration. Rare:

	pulmonary reactions, peripheral neuropathy, hepatic reactions.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the

responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
25/8/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date
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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Trimethoprim

for the treatment of

Urinary Tract Infection (UTI) in women aged 16–64 years

Healthcare professionals covered and training

Patient Group Direction for the administration of Trimethoprim for the treatment of UTI in women aged 16-64	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Trimethoprim is indicated for the treatment of uncomplicated lower urinary tract infection (UTI) in non-pregnant women aged 16 to 64 years, presenting with symptoms such as dysuria, frequency, and urgency, in the absence of systemic signs of infection.
Inclusion criteria	- Women aged between 16 and 64 years. - Presenting with typical symptoms of uncomplicated lower UTI (e.g., dysuria, urgency, frequency). - Able to give informed consent. - No signs of systemic infection (e.g., fever, flank pain). - Not currently pregnant or breastfeeding.
Exclusion criteria	- Known hypersensitivity to trimethoprim or any excipients in the formulation. - Pregnant or breastfeeding. - Known folate deficiency or megaloblastic anaemia. - Significant renal or hepatic impairment. - Use of interacting drugs (e.g., methotrexate) without clinical review. - Symptoms lasting more than 7 days or history of recurrent UTIs.
Cautions	- Consider alternative diagnosis if symptoms are atypical or if vaginal discharge is present. - Advise taking the medication at evenly spaced intervals. - Monitor for signs of hypersensitivity, blood disorders or gastrointestinal upset.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Trimethoprim 200 mg tablet
Legal category	POM
Storage	Store below 25°C in a dry place
Route/method of administration	Oral, with or without food
Dose and frequency	200 mg twice daily for 3 days
Quantity to be administered and/or supplied	6 tablets (3-day course)
Maximum or minimum treatment period	3 days; reassess if no improvement or if symptoms recur
Adverse effects	Nausea, headache, flatulence, diarrhoea, and rash, pruritus, nausea, vomiting. Rare: blood dyscrasias (e.g., megaloblastic anaemia), hypersensitivity reactions, liver enzyme abnormalities.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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